Published access is strong, with a concentrated gap

BELSOMRA compared with DAYVIGO and QUVIVIQ across US standalone Medicare prescription drug plans.

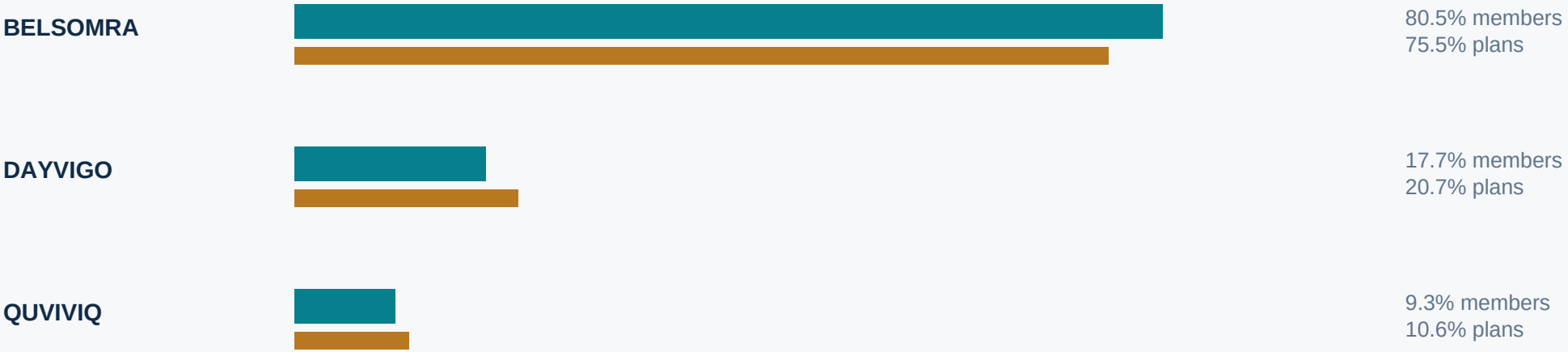
367
standalone PDPs

80.5%
BELSOMRA weighted listing

70
plans in investigation queue

3.21M
associated plan members

Plan coverage and member coverage answer different questions



Teal: enrollment-weighted listing. Amber: share of plans listing any catalogued strength. Coverage overlaps across brands; percentages must not be added. Listing is not a guarantee of claim payment.

Read listings and restrictions separately

Each brand has a complete strength catalogue. A plan is counted once per brand and snapshot.

Brand	Listed / known plans	Members in listed plans	No PA / ST plans	QL plans
BELSOMRA	277 / 367	15.25M	271	277
DAYVIGO	76 / 367	3.36M	70	76
QUVIVIQ	39 / 367	1.76M	39	39

What the data supports

BELSOMRA is listed by 277 of 367 plans, associated with 15,253,298 members. Its member-weighted listing exceeds its plan listing rate by 5.0 percentage points.

What still needs investigation

Quantity limits apply to every listed plan-brand in the real case study. A plan without prior authorization or step therapy can still have a quantity limit. Tier values describe a plan benefit design; they are not universal prices or clinical rankings.

A higher weighted rate does not prove broader access

Compare 2026-01 with 2026-08. August enrollment is held fixed in the paired-cohort comparison.

Brand	January actual weights	August actual weights	Jan with Aug weights	Aug with Aug weights
BELSOMRA	79.54%	80.51%	80.51%	80.51%
DAYVIGO	18.80%	17.75%	17.75%	17.75%
QUVIVIQ	9.98%	9.31%	9.31%	9.31%

BELSOMRA: +0.97 percentage points with each month's enrollment

Listing sets are unchanged across the compared plans. With the same August weights at both dates, the listing rates are identical. The changing observed weighted rates therefore reflect enrollment composition within this cohort, not listing expansion.

BELSOMRA strength-listing loss plans: 0. Added flags across compared strengths: PA 0, ST 0, QL 0. No events is a valid analytical result.

Start with the largest documented access differences

The queue is a request for payer-document review. It does not estimate lost or recoverable sales.

Contract organization	Plans	Associated enrollment
SILVERSCRIPT INSURANCE COMPANY	33	1,822,653
MEDCO CONTAINMENT LIFE AND MEDCO CONTAINMENT NY	33	1,197,322
ANTHEM INSURANCE CO. & BCBSMA & BCBSRI & BCBSVT	2	174,524
USABLE MUTUAL INSURANCE COMPANY	2	17,555

Top two contract organizations account for 94.0% of queue enrollment.

1. Check current formulary documents and indication / utilization criteria.
2. Review whether the difference persists across all strengths.
3. Discuss access barriers using plan-level evidence; do not infer clinical suitability or sales impact.

Provenance: 2,244,023 original formulary rows scanned across two snapshots; archive and member-file SHA-256 hashes retained. Four unkeyed enrollment records were quarantined. The twenty-case source audit is saved separately. These pages are Python-generated, not Desktop screenshots.